Flobotics

State of RCM Automation 2026 Edition

AI giants in RCM, WISeR pilot across states, fewer paying ACA enrollees... 2026 introduced a lot to revenue cycle in U.S. healthcare. Here's our take on legal & regulatory changes, finance & payments shifts and automation tech trends, all topped with 4 bets for 2027.



SOURCES AND TOOLS COVERED IN THIS REPORT

CMS.gov · U.S. Department of Justice · Medicaid.gov Keeping America Healthy · GovInfo · ONC Office of the National Coordinator for Health Information Technology · Morgan Lewis · Holland & Knight · HLB Hooper Lundy Bookman · America's Essential Hospitals · Becker's Hospital Review · Fierce Healthcare · HealthcareDive · The HIPAA · Guidehouse · hfma · Market Scan · XiFiN · Black Book Research Insights · auxis · AKASA · experian health · DataSpring · AMA · aws · UiPath

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Report summary

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Legal & regulatory

WISeR vs Providers

In January, CMS began a **prior authorization pilot in traditional Medicare** in six states (AZ, NJ, OH, OK, TX and WA).

WISeR initiative aims to reshape the cash flow within revenue cycle management (RCM): vendors are paid a share of the savings from denials that stand on appeal, every denial needs a clinician's review, and CMS recoups the payment if a denial is overturned.

In May, GAO ruled that the WISeR notice counts as a rule under the Congressional Review Act, and the model was not stopped.

On top of that, six states (AL, CO, GA, IA, UT and WA) passed **laws that limit artificial intelligence (AI) usage in coverage decisions** and enforce a human-in-the-loop approach among insurers.

Our take:

WISeR model seems to be putting more work and pressure on the provider's side than on the payer's.

Since WISeR is set to run through 2031, one question arises:

In what automation should providers invest to survive?

Read more → ·p.9

Automation tech

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New AI tools review

Between January 7 and March 5, 2026, Anthropic, Microsoft, AWS and OpenAI each launched or expanded healthcare AI.

Even UiPath followed with agentic solutions for healthcare, including a prior authorization tool built with Genzeon, one of the six vendors CMS selected for the WISeR model.

Some features of these tools are still **in preview or beta** and most likely stay that way for long.

Our take:

Each is useful, but none is a market-ready self sufficient revenue cycle automation system. We believe that AI will not fully replace single-process robotic process automation (RPA) in revenue cycle management.

Interested in details?

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3 Automation ≠ ROI

Coverage is shrinking faster than payment rates move. **Paid ACA enrollment fell 13%** (19.2M vs 22.1M, ASPE), and hospitals already see the effect: HCA's uninsured admissions rose about 27% in Q3, while exchange admissions fell about 19%. More **self-pay patients mean more bad debt** for hospitals that keep treating them.

On the rate side, the proposed ASP minus 33.4% for 340B-acquired drugs would raise **revenue pressure for 340B hospitals** from Jan 1, 2027, if finalized. Generating savings by investing in automation has become a standard survival tactic for the U.S. healthcare revenue cycle management teams.

Our take:

In 2027, tools will matter less than partners. RCM teams will have to tell who stays after the demo from who disappears, and pricing tied to results from pricing tied to promises or an implementation calendar.

Read more → ·p.10

4 Our bets for 2027

Black Book (Mar 2026) expects AI automation within RCM to grow at a **high double-digit pace** through 2027.

Where the work will sit is disputed. Auxis/SSON (2025) found **90% of GBS respondents in Latin America** or planning to be within three years, while TempDev (Apr 2026) sees organizations **bringing RCM back in-house**.

Auxis sells nearshore and TempDev sells AI, so **each forecast points where its author earns money**.

Our bets:

- **Offshoring to Asia will shrink.**
- **Nearshoring to LATAM will lose its cost edge.**
- **Automation will become RCM bedrock.**
- **Artificial intelligence won't entirely replace RPA.**

Main events of 2026

Legal & regulatory · Finance & Payments
Quarter by quarter

Legal & regulatory

In 2026, prior authorization moved into rules and into traditional Medicare, enforcement kept targeting Medicare Advantage coding, and vendor incidents became revenue cycle incidents.

Q1:

(Jan 14) Kaiser and Aetna settle. Kaiser agreed to $556M, and Aetna to $117.7M (Mar 11), to settle Medicare Advantage diagnosis-coding allegations. **(Jan 15)** WISeR starts. WISeR is a CMS test model that brings prior authorization into traditional Medicare, which had almost none. In six states (AZ, NJ, OH, OK, TX and WA), technology vendors review selected services, such as skin substitutes and knee arthroscopy, before the service. It is set to run to 2031 and the vendors are paid from savings. **(Feb 25)** OIG audits a hospital. Auditors reviewed 100 Medicare claims from 2020 and 2021 and found billing errors in 26 of them, worth $272,364 in net overpayments. Extrapolated to the full claim population, OIG estimates at least $12.1M and asks Sarasota Memorial Hospital for a refund. The hospital disputes most of the findings. **(Mar 16)** Vendors disclose incidents. CareCloud and NYC Health + Hospitals (notice Mar 24) reported incidents at vendors, not at hospital networks. Black Book reports that most hospitals hit by a major clearinghouse disruption saw an immediate cash flow impact. **(Mar 23)** Washington signs its AI law. It is one of six states (AL, CO, GA, IA, UT and WA) that passed laws in 2026 limiting how insurers use AI in coverage decisions and enforcing a human-in-the-loop approach. The law took effect on Jun 11, Alabama's takes effect on Oct 1, 2026, and Colorado's, Georgia's and Utah's on Jan 1, 2027.

Q2:

Plans report fewer prior auths. AHIP and BCBSA plans reported 11% fewer prior authorizations. The figure is plan-reported. **(Apr 6)** Medicare Advantage 2027 rate final. CMS set +2.48%, against +0.09% proposed in January. **(May 12)** GAO rules on WISeR. GAO treats the WISeR notice as a rule under the Congressional Review Act. The model keeps running. **(Jun 3)** Matrix Medical settles. Matrix Medical Network, HealthFair and its founder agreed to $56.5M over Medicare Advantage diagnosis-code allegations. **(Jun 8)** OIG reports on Medicare Advantage appeals. 95% of appealed skilled-nursing denials were overturned. **(Jun 23)** Takedown names 455 defendants. The national health care fraud takedown covered over $6.5B in alleged losses.

Q3:

(Aug 26) Villages Health System settles. It agreed to $541.5M to resolve self-disclosed Medicare Advantage diagnosis-code allegations. Coding intensity is under scrutiny from both sides: OIG audits (Sep 15) estimated $130.9M at Humana and $46.9M at UnitedHealthcare of Wisconsin. BCBSA claims about $942M in excess cost from AI-assisted hospital billing; neither claim has been independently audited. This means AI-assisted coding needs auditable human review. **(Sep 17)** New York court bars IDR suits. It held that the No Surprises Act gives no private right to enforce IDR awards, so disputes are fought case by case. **(Oct 1)** UHC cuts prior authorization. UnitedHealthcare removed about 1,700 codes and says 2% of services need it. Prior authorization is easing at one large payer, not everywhere: in pathology, XiFin saw prior authorization denials fall 42.3% as payors eased rules for tests with an 80% or better approval chance, while medical-necessity denials rose.

Q4:

(Oct 23) Prior authorization pledge. CMS and Labor: Google, Amazon, Meta, Oracle and Axon will adopt electronic prior authorization in self-funded plans. **(Late Oct)** WISeR evidence. A JAMA study (Epic Cosmos data) found epidural steroid injections about 15% and other targeted services about 20% lower than in non-pilot states. The effect faded from Q1 to Q2. **(Oct 30)** CommonSpirit and Conifer. CommonSpirit's service with Conifer ends. **(Nov 1)** IDR batching. New batching rules apply to disputes opened from this date. **(Nov 2)** Conifer layoffs. Conifer layoffs begin. **(Dec 7)** OIG prior authorization update. CMS is expected to update on OIG prior authorization recommendations. **(Dec 7)** Price transparency. The rule CMS-9882-F takes effect.

Finance & Payments

Coverage shrank before any payment rule did, rate updates then moved margin by service line, and the cost of the coverage shift started to show up in paid enrollment and write-offs.

Q1:

(Jan 26) Medicare Advantage proposal. The CY2027 advance notice proposed a 0.09% increase. **(Mar 20)** New York waiver ends. The end of the waiver changed payment rules mid-quarter. **(Mar 27)** ACA enrollment falls. 23.1M people selected a 2026 plan (final report), about 5% below 2025, and new consumers fell 13% to 3.6M. This means more self-pay patients and more bad debt later in the year.

Q2:

(Apr 6) Medicare Advantage rate final. The CY2027 final was 2.48%, worth over $13B, or 4.98% with the coding trend, a larger payment and a larger audit target. **(Apr 7)** Guidehouse and HFMA survey. 66% of providers outsource all or part of their revenue cycle. Only 12% rank cost to collect among their top three stressors, against 25% a year earlier. **(Apr 10)** Inpatient update proposed. The proposed FY2027 IPPS update was 2.4%; it was finalized in Q3. **(Jun 8)** OIG reviews appeals. In a one-month review of 19 plans, 95% of appealed Medicare Advantage skilled-nursing denials were overturned. This means appeal capacity decides revenue as much as first-pass approval. XiFin data from labs and pathology show appeal success of 60% for contracted Medicare plans against 5% for non-contracted Medicaid plans. **(Jun 26)** Paid ACA enrollment falls. Effectuated enrollment in February 2026 was 19.2M, against 22.1M in February 2025 (–13%). 83% of plan selectors paid in 2026, against 91% in 2025 (CBPP).

Q3:

(Jul 2) Outpatient rule proposed. The CY2027 proposal would pay 340B-acquired drugs at ASP minus 33.4%. **(Jul 14)** Physician fee schedule proposed. Conversion factors are $33.17 and $32.84. **(Jul 31)** Inpatient update final. CMS finalized 2.3% (3.2% market basket less a 0.9-point productivity adjustment). This means inpatient payment grows 0.9 points slower than the market basket of hospital costs, which squeezes margin. **(Aug 31)** CMS cancels enrollments. About 315,000 enrollments covering more than 760,000 people were cancelled, citing unverified broker enrollments. Enhanced ACA subsidies expired on Dec 31, 2025 and no extension is law. This means coverage loss converts to self-pay and charity care, which changes cost-to-collect and staffing needs more than a 2% rate update does.

Q4:

(Oct 1) Inpatient rule takes effect. The FY2027 inpatient payment rule is in force. **(Oct 22)** Recoupment survey. A Waystar and Modern Healthcare survey found 42% of hospitals face at least $1M a year in recoupment requests (vendor-run). **(Oct 27)** HCA reports Q3. Uninsured admissions rose about 50% in Q2 and about 27% in Q3, while exchange admissions fell about 19%. Uninsured patients were 10.5% of volume, against 8.5%, a shift worth about $400M of pretax income. Tenet's exchange admissions and revenue fell 18%. **(Oct 28)** Denials keep rising. 75% of providers report rising denials (Experian), while measured hospital initial denial rates fell in H1 2026 (Kodiak). In H1 insurers also clawed back more paid claims (1.57% vs 1.38% of accounts receivable), and providers recovered 12.2% fewer dollars on appeals after payment (Kodiak). We expect margins to stay under pressure in 2027. **(Oct 30)** CommonSpirit and Conifer. CommonSpirit's service with Conifer ends. **(Nov 1)** Open enrollment starts. Benchmark premiums rise 11% in HealthCare.gov states and 16% elsewhere. Humana exits Medicare Advantage plans covering about 600,000 members. **(Nov 2)** Conifer layoffs. Conifer layoffs begin. **(Early Nov)** Final rules expected. The CY2027 outpatient and physician fee schedule final rules usually arrive in early November. **(Dec 7)** OIG prior authorization update. CMS is expected to update on OIG prior authorization recommendations. **(Dec 11)** Federal funding expires. It ends under the continuing resolution; Medicare-dependent hospital and low-volume adjustments expire on Dec 31.

Our take on 2026

New AI Tools for RCM · AI vs RPA vs API
Future Delivery Models · Our Bets for 2027

Brand new AI tools

In Q1, five large technology companies (Anthropic, OpenAI, UiPath, Microsoft and AWS) launched or expanded AI tools aimed at healthcare. Each has big potential but is far from being a complete revenue cycle automation system. Here are our impressions on all 5:

Claude for Healthcare, launched at the J.P. Morgan Healthcare Conference in January, connects Claude to U.S. medical data infrastructure through connectors to the CMS Coverage Database (LCD/NCD), ICD-10, the NPI Registry and PubMed. HIPAA coverage was not automatic: it needed a signed business associate agreement (BAA) and an Enterprise plan (a seat fee plus usage at application programming interface (API) rates, with HIPAA-ready seats at an extra cost), on which an administrator had to switch it on. We tested Claude's capabilities at one of our clients, a private clinic from Miami. For revenue cycle teams the nearest return is in prior authorization, appeals and coding.

ChatGPT for Healthcare, launched in January, is aimed to be a shared workspace for clinicians and administrators, with a BAA available to eligible customers. It drafts but does not execute: OpenAI lists no connection to eligibility feeds, payer fee schedules or clearinghouse status. We tested it internally but we see a narrow use for it in revenue cycle teams. It offers reusable templates, among them prior authorization support, drafts of administrative documents, and answers aligned with the organization's own policies through SharePoint and similar tools. OpenAI publishes no price; it is tied to ChatGPT Enterprise.

UiPath launched agentic solutions for healthcare at ViVE on February 23, with undisclosed pricing: medical records summarization, claim denial prevention and resolution, and prior authorization, the last built with Genzeon, one of six vendors CMS selected for the WISeR model. Since July, developers can also build UiPath agents from Claude Code, Cursor or Codex CLI (generally available since September 16). We are a certified UiPath partner and will continue to test the usability and return on investment (ROI) of their solutions as suitable for the RCM niche in the near future.

Microsoft Dragon Copilot is a documentation tool, not a billing tool: it turns the visit conversation into a clinical note and suggests ICD-10 specificity, but does not check medical necessity or flag hierarchical condition category (HCC) gaps, covers neither the front desk nor patient data intake, and the physician stays liable for the note. We expect revenue gains only where coders, documentation specialists and denial feedback are redesigned around AI-written notes. Microsoft publishes no list price; licenses are per user or Flex, with ambient AI pay-as-you-go.

AWS Connect Health is, in our reading, an infrastructure play: it sells building blocks, not a revenue cycle system: prebuilt agents for patient verification, scheduling, patient insights, documentation and coding. AWS aims it at providers and health technology developers, and its agents plug into Amazon Connect, so the natural buyer already runs on AWS. At launch on March 5 only verification and documentation were generally available, the rest in preview. AWS lists no claim scrubbing, payer-specific rules, denial workflows or 835 remittance handling. Pricing: $99 per user per month for ambient documentation and $0.15 per patient verification conversation. After careful consideration and research we decided not to continue exploring AWS Connect Health and went for a more flexible approach, Python-based automations for our clients.

With so many changes on the market, testing is key. That is why we launched our AI Lab, where we test where AI stops working, where classic RPA still wins and which new tools (like Google Antigravity and Cursor) hold up on real revenue cycle work. For us, the choice of tool is a matter of scope, and the right decision decides the ROI or wasted investment, time and money.



Karl Mielnicki
CTO & Co-Founder, Flobotics

Not every process needs agentic AI

Start with a goal. Then apply the technology.

The level of automation alone is not enough to measure success. The real goal is to aim improvements at achieving certain business outcomes, such as lowered cost to collect, higher collection rate, or unlocked full-time equivalent (FTE).

What matters is the process, not the tool, and that is no argument against artificial intelligence. Where the work is unstructured, such as faxes, handwriting and payer rules that change, agents earn their place, and we build them.

However, it's worth noting that RPA is still valid. In our experience it is enough where the input is structured and the rules are stable, as with eligibility checks. Black Book found RPA in at least one revenue cycle function at 21% of provider organizations in 2025, and 83% expected to extend it to denial management, prior authorization and financial clearance by late 2026. In June 2026 we freed NEPDental counter staff from nearly 70 eligibility checks a day with a single robotic process automation written purely in Python. At a pain management network (PT-CoA), automating eligibility checks cut a check from 5–7 minutes to about 40 seconds and freed up capacity equal to about 3 FTEs.

Choosing an automation partner is slow, expensive and sometimes scary. In FinThrive's 2026 survey of 100 hospital finance leaders, nearly 60% plan to consolidate revenue cycle management vendors within three years.

That is why the discovery step matters: mapping the process and finding what is worth automating before any technology is chosen. We believe that, done properly, it significantly improves the likelihood of positive ROI, no matter the technology. Keeping this in mind is very important, since Medicare Advantage is starting to look like Medicare "Disadvantage"... KFF counted about 2.6 million enrollees in plans terminated for 2026, twice the 1.3 million of a year earlier, and a Johns Hopkins study in JAMA puts forced disenrollment at 10% in non-employer HMO and PPO plans, against just over 1% a year in 2018 to 2024. Insurers pulled out, margins collapsed, plans disappeared, and RCM teams deal with the mess: more denials, more complexity, less predictability. We expect every plan change to mean eligibility re-verified and payer rules relearned, work that lands on the RCM team. This is the environment where AI and automation stop being optional, and where the order of the steps matters. We believe the useful question is not whether AI can do a process, but which business outcome it should move and which tool gets there most cheaply.

	RPA	API	AI AGENT
Best for:	Fixed steps in portals and electronic health record (EHR), stable rules, structured input	Moving data between systems that expose an interface	Unstructured or variable input that needs judgment
Example:	Eligibility checks: nearly 70 a day at NEPDental, one Python RPA	Syncing verified coverage between an EHR and a billing system	Prior authorization with payer-specific questions: 10–15 minutes down to 1–2 per packet in one Flobotics project



Bart Teodorczuk
Lead AI & Automation Advisor, Flobotics

Four ways to buy automation

Headcount-based outsourcing, fixed-fee, subscription or transactional, and what the AI Discovery stage will change.

Introduction

In 2026 the pricing model decides who carries the risk of an automation project, and so who has a reason to make it work. Besides time-based billing, which most buyers still know, four models are in use: outsourcing priced by headcount, fixed-fee, subscription and transactional. We judge each by staffing, margin, processes, ownership and key performance indicators (KPIs).

Four models, four trade-offs

Outsourcing, priced by headcount. An external team takes over a defined part of the revenue cycle and works as an extension of the provider's staff, either offshore, with the team in a distant country such as in Asia, or nearshore, with the team in a nearby time zone such as in Latin America. We expect both to lose ground, because AI automation changes why work was outsourced. The plus is capacity without hiring, with recruiting and training left to the vendor. The minus is dependence. Process knowledge sits with the vendor's people, cost scales with headcount and not with results, and unless the contract says otherwise, the gains from any automation the vendor builds stay with the vendor.

Fixed-fee. A set price for a defined scope. The plus is a known budget and an easy approval. The minus is that scope rarely stays still. In 2026 payer rules, tools and vendor features change within months, so a scope written in January can be out of date by summer, and the price rises through change orders or a built-in risk margin.

Subscription. A recurring fee for access to a tool or a team, whatever the volume. The plus is predictable cost, quick adoption and updates included as the tools change. The minus is that the fee is paid whether or not the tool is used, and the value depends on how well it fits the provider's own workflow and on whether staff adopt it.

Transactional. Payment per unit of work, such as a claim or an eligibility check. The plus is a low entry cost and a price that follows output. The minus is that the vendor is paid for volume, not for a resolved problem, so a cheap unit can hide rework, and spend is hard to forecast when volumes swing.

Why billing by the hour no longer makes sense

Automation exists to remove hours of manual work. Hourly billing is outdated for work with measurable outcomes. Hours also measure effort and not outcome: the buyer pays the same whether the denial rate falls or not, and has little way to check whether the hours were needed. Under time-based billing, the better the automation, the fewer hours the vendor can bill, so the vendor is paid less for doing the job well. We expect AI tools to shorten the time it takes to build and change an automation, so the hours behind a deliverable shrink while its value to the provider does not.

Our approach

We see cost to collect and collection rate as better business goals than the level of automation.

Meetings with clients, including on site, are valuable because they help us understand their goals, KPIs and business.

Our delivery team is a UiPath developer, a Python developer and a consultant. The team runs shadowing sessions, maps current work and helps build the right habits for using the tool. The offer covers automation, consulting, training, and building data systems and dashboards.

We are also developing an agentic system to support delivery, meant to cover analysis, process mapping, implementation and maintenance, with deployment planned soon. We intend to move gradually from hand-built UiPath workflows and low-code toward this system.



Krzysztof Szwed
CDO, Flobotics

Our bets for 2027

Here are 4 bets for the upcoming year from our perspective.

Our bets for 2027

1. Offshoring to Asia will shrink.
2. Nearshoring to LATAM will lose its cost edge.
3. Automation will become RCM bedrock.
4. Artificial intelligence won't entirely replace robotic process automation.

In our view, automation vendors are the big winners of 2026. We are closing this year with over 140% of last year's financial result, entirely with existing clients. Truth be told, we've never seen so much interest in RCM automation as during this year's ViVE digital health conference edition in LA.

Unfortunately, we still expect providers to have less money. We expect reimbursement pressure and payer-mix shifts to compress provider margins. Patients are in a worse spot too. With the ACA subsidies lapsed, they pay higher premiums, so more of the bill falls on them (self-pay), and hospitals will not collect part of it, which ends up as bad debt. Under that pressure, the question is no longer where the work is cheapest, but whether it needs to be done by hand at all.

That's why we also expect offshoring to Asia and nearshoring to LATAM both to lose ground. FinThrive's 2026 survey already found that over 70% of respondents expect to reduce reliance on third-party revenue cycle vendors, and nearly 60% plan to consolidate RCM vendors within three years.

AI won't entirely replace RPA. We expect AI to significantly increase the capacity of revenue cycle management teams, because routine work gets done faster and staff have more time for patients. But it will not eliminate the human-factor bottleneck. Someone still has to review exceptions, talk to patients and payers, and take responsibility for decisions that carry compliance risk. We also expect RPA to stay where it already works best. Where input is structured and rules are stable, such as a standard eligibility check, we believe a rule-based robot is cheaper and more predictable than an AI agent, so RPA is still enough. We expect AI agents to take over the unstructured work, such as reading denial letters, clinical notes and payer correspondence, and the two to run side by side.

Main events planned for 2027

DATE	WHAT HAPPENS
Jan 1, 2027	CMS-0057-F: payers must offer electronic prior authorization APIs. We expect fewer faxes and re-keying, but only with payers that go live.
Jan 1, 2027	Medicaid work requirement starts, with 80 hours a month of work, service or school, or income of at least $580 a month. We expect more patients losing coverage, so more self-pay and bad debt.
Jan 1, 2027	Medicaid renewals every six months for expansion adults. We expect more coverage gaps and claims denied for inactive coverage.
Jan 1, 2027 (targeted)	340B rebate pilot (Medicare-negotiated drugs only) and the AHIP and BCBSA electronic prior authorization framework are due to start. We expect tied-up cash on 340B drugs, and less phone work only if payers deliver.
Jan 1, 2027	State AI utilization-review laws in Utah, Colorado and Georgia (date per the bill text): a clinician must review AI-based denials. We expect fewer purely automated denials and a clearer paper trail.
~Feb 1, 2027	ACA broker moratorium expected to end, after about 315,000 cancelled enrollments. We expect more eligibility denials and self-pay.



Filip Nasiadko
CEO & Co-Founder, Flobotics

Method and evidence grades

METHOD IN BRIEF

Each fact was checked against a primary record where one exists: CMS fact sheets, the Federal Register, DOJ, HHS-OIG, court opinions, company filings. Becker's newsletters were used only as a signal for what to verify. Sponsored webinar promotions were excluded.

Cut-off: November 1, 2026. **Passes:** three independent verification passes. **Limits:** Kaufman Hall report PDFs and Congress.gov were not accessible; state AI-law dates come from secondary sources.

EVIDENCE GRADES USED IN THE REFERENCES

P	**Primary record:** rule, filing, agency release, court opinion
I	**Independent or association survey:** government-funded, HFMA, CAQH, AMA
V	**Vendor- or industry-funded:** Experian Health, AKASA, Kodiak, XiFin, Black Book, BCBSA
S	**Secondary press or summary:** Becker's, Fierce Healthcare, Healthcare Dive, law-firm alerts
E	**Opinion or forecast:** Flobotics commentary, projections, scheduled-date forecasts

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5	CareCloud 8-K, sec.gov – CareCloud Form 8-K Item 1.05	P
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7	Black Book Research: AI-Driven RCM Managed Services, Claims Optimization, Denial Prevention and Revenue Protection, March 2026 – Black Book Market Research LLC; PDF in the Library; reproduction restricted, findings paraphrased	I
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24	Becker's Hospital Review; Fierce Healthcare (company statements) ↗ fiercehealthcare.com	S
25	XiFin 2025 Payor Denial Impact Report: 2024 data, over 20 million claims – XiFin, Inc. white paper (25-WP-192); PDF in the Library	V
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About the authors

The people behind the commentary



Filip Nasiadko

CEO & Co-Founder

The industry dreamer who's staking his own company on where revenue cycle is headed next. He has skin in the game most commentators don't, so when he calls a trend, it's a bet he's placing with real money.



Karl Mielnicki

CTO & Co-Founder

Kaizen-minded process improver who tracks every wasted dollar in your revenue cycle. He thinks like a CFO, not a salesperson, so his ROI numbers survive an actual finance review.



Bart Teodorczuk

Lead AI & Automation Advisor

A hands-on advisor who tests the models and stacks himself, so he knows which vendor claims hold up under real volume.



Krzysztof Szwed

CDO

The operations realist who assumes your rollout will break and tells you exactly where. He cares about whether your staff can absorb the workload shift without burning out or blowing an SLA.

Nothing here is legal or financial advice.

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Jędrzej Szymula

Content Manager